Bleeding after birth

PPH prevention and diagnosis

Provide respectful care **Prevent infection**

Prepare for birth
Check equipment and medications

Baby born

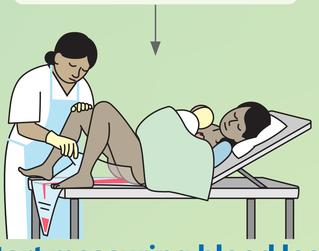
Give uterotonic within 1 min

Oxytocin 10 IU IM or IV
or Carbetocin/HSC 100 µg IM or IV
or Misoprostol 400-600 µg PO

Assess and record

- Uterine tone
- Blood loss and flow
- BP and pulse

RECORD EVERY 15 MIN



Start measuring blood loss

Cut the cord between 1–3 min
Perform controlled cord traction

Be alert for PPH

- ≥ 300 mL and warning sign
- or
- ≥ 500 mL
- or
- Clinical judgment

PPH? START BUNDLE

Placenta out?

- Out → **Check tone**
  Massage if soft
  Check bleeding
- Not out after 15 min → **Empty bladder**
  Repeat controlled cord traction
  Placenta out?
  - Out → **Check tone**
    Massage if soft
    Check bleeding
  - Not out after 30 min → **Remove manually**

Check placenta

- Incomplete → **Remove manually**
- Complete → **Check for tears and repair**

Concerns?

- No → **Check for tears and repair**
- Yes → **Seek advanced care**



Continue care
Monitor bleeding
Check tone and vital signs
Encourage breastfeeding

RECORD EVERY 15 MIN

RECORD EVERY 15 MIN

PPH? START BUNDLE

PPH? START BUNDLE

For more information, please contact:
Department of Sexual, Reproductive, Maternal, Child, Adolescent Health and Ageing
World Health Organization, Geneva, Switzerland
20 Avenue Appia
1211 Geneva 27
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Email: srhhrp@who.int
Website: https://www.who.int

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FIGO International Federation of Gynecology and Obstetrics
International Confederation of Midwives
Laerdal helping save lives
UNFPA
human reproduction programme hrp research for impact UNDP-UNFPA-UNICEF-WHO-WORLD BANK
World Health Organization

Bleeding after birth

PPH treatment

Provide respectful care **Prevent infection**

PPH diagnosed



Call for help
Start PPH bundle



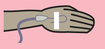
Massage uterus



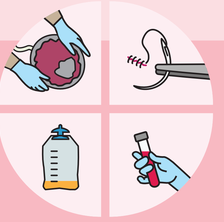
Oxytocic
Oxytocin 10 IU **or**
Misoprostol 800 µg



TXA
1g IV



IV Fluids



Examine and address causes

15 min

Still bleeding?

No

Yes ---------- if needed ----------►



Manage shock



Arrange transfer / surgery



Apply NASG

Escalate care
Communicate
Assign roles

Oxytocin 20 IU in 1000 mL IV
or Misoprostol 800 µg SL
or Ergometrine 200 µg IV or IM



Continue massage



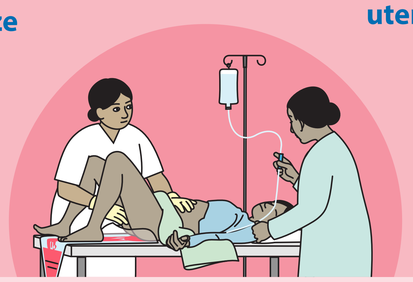
Start 2nd IV



Catheterize bladder



Give additional uterotonics





Remove clots and debris



Repeat TXA



Assess and record

Consider if needed



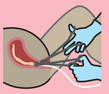
Safe blood transfusion



Repair cervical tears



Bimanual compression



Balloon tamponade



Compress aorta



Post-PPH care
Assess and record

Uterine tone
Blood loss and flow
BP and pulse

Every 15 min x 4
Then every 30 min x 4

← No — **Still bleeding?** — Yes →

Surgery and post-surgical care

For more information, please contact:
Department of Sexual, Reproductive, Maternal, Child, Adolescent Health and Ageing
World Health Organization, Geneva, Switzerland
20 Avenue Appia
1211 Geneva 27
Switzerland
Email: srhhrp@who.int
Website: https://www.who.int

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